

Occupational English Test

WRITING SUB-TEST: MEDICINE

TIME ALLOWED: READING TIME: 5 MINUTES
WRITING TIME: 40 MINUTES



Read the case notes and complete the writing task which follows.

Notes:

Emma Johnson is an 11-year-old patient in your general practice.

Patient history

30/11/09

History: Nocturnal cough last 2 wks associated with runny nose and mild fever
Previously well
Family history of asthma – mother & older sister aged 15
Past history – nil
Medications – nil
Allergies – none known

Examination: Slightly flushed, temp 37.8°C
Red pharynx, ears normal
Chest – no obvious wheeze, no focal signs
Peak expiratory flow rate (PEFR) – 250 L/min (expected 350 L/min)

Assessment: Viral upper respiratory infection
Possible associated asthma

Plan: Paracetamol, fluids for infection
Review if worsens

21/12/09

Examination: Patient returns with persistent nocturnal cough. Runny nose and fever now completely resolved. Cough also noted on exercise.
Chest auscultation – faint basal wheeze, PEFR 230 L/min
Ears, nose and throat normal

Assessment: Probable asthma

Plan: Trial inhaled salbutamol – Ventolin puffer 2 puffs t.d.s.
Keep record of morning and evening PEFR readings
Review 3 wks

11/01/10

Examination: Review symptoms slightly improved; exercise tolerance better
Less nocturnal disturbance
Chest auscultation – no wheeze heard
PEFR 280 L/min

Assessment: Improved but asthma still slightly unstable

Plan: Commence inhaled corticosteroid – Becotide 200 2 puffs t.d.s.
Continue Ventolin as required
Continue PEFR monitoring
Review 4 wks

8/02/10

- Examination:** Much improved; minimal nocturnal & exercise-induced symptoms
Chest auscultation – clear, PEFR 340 L/min
- Assessment:** Well-controlled asthma
- Plan:** Maintain on current therapy
Review 2 mths

10/04/10

- Examination:** Worsening shortness of breath & wheezing over last 48 hrs w. runny nose, fever, loss of appetite
Red throat, ears normal
Obvious difficulty in breathing with use of accessory muscles
Chest auscultation – widespread wheeze, no focal signs
PEFR 140 L/min
- Assessment:** Severe asthma triggered by viral upper respiratory infection
- Plan:** Ventolin nebuliser statim – minimal relief
Urgent assessment at hospital

Writing task:

Using the information given in the case notes, write a letter of referral to Dr B Townsend in the Emergency Department at the Newtown Children's Hospital.

In your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180 - 200 words.